

ADVERSE DRUG REACTION (ADR) REPORTING AND DOCUMENTATION

Completed Sample Report — For Academic Submission

Note: The patient/case details below are a fictional educational case prepared for assignment purposes.

1. PATIENT INFORMATION

Patient initials / ID	R.K. / ADR-2026-001
Age / Sex	32 years / Female
Weight	58 kg
Relevant medical history	No significant past medical history; no known chronic disease reported.
Known allergies / previous ADR	No known drug allergies; no previous ADR reported.

2. ADR DETAILS

Suspected reaction	Generalized urticaria (hives), itching and mild facial swelling.
Date of onset	12 September 2026, approximately 2 hours after medicine administration.
Date resolved	13 September 2026 after withdrawal of suspected medicine and treatment.
Seriousness	Medically important event; patient was evaluated in the emergency/outpatient setting. No hospitalization required.
Outcome	Recovered.
Clinical description	Patient developed generalized itchy red wheals with mild facial swelling. No loss of consciousness or respiratory distress was reported.

3. SUSPECTED MEDICINAL PRODUCT

Drug / Brand	Dose & Route	Frequency	Therapy dates	Indication
Amoxicillin 500 mg capsule	500 mg, oral	Three times daily	12–12 Sep 2026	Suspected bacterial throat infection
Paracetamol 500 mg tablet	500 mg, oral	As required	10–12 Sep 2026	Fever / pain
Cetirizine 10 mg tablet	10 mg, oral	Once daily	12–13 Sep 2026	ADR treatment

4. CONCOMITANT MEDICINES / OTHER EXPOSURES

Concomitant medicines	Paracetamol 500 mg as required for fever. No herbal products, OTC medicines or vaccines reported during the relevant period.
Food / environmental exposure	No new food, cosmetic or environmental exposure reported.

5. CLINICAL INVESTIGATION & MANAGEMENT

Relevant examination	Generalized urticaria and itching with mild facial edema. Vital signs stable; no wheezing or respiratory distress.
Laboratory / diagnostic results	No specific laboratory abnormality identified. Clinical assessment supported an acute hypersensitivity-type reaction.
Treatment given	Suspected medicine discontinued. Cetirizine 10 mg orally was administered; patient observed until symptoms improved.
Dechallenge	Positive — symptoms improved after withdrawal of amoxicillin and treatment.
Rechallenge	Not done — re-exposure was avoided for patient safety.

6. CAUSALITY ASSESSMENT

Suspected drug	Amoxicillin 500 mg capsule.
Assessment	Probable / Likely.
Reason	Temporal relationship was compatible, symptoms are consistent with a known hypersensitivity reaction, and improvement occurred after withdrawal. Rechallenge was not performed.
Alternative causes	No new food, cosmetic or other medicine exposure was identified; however, complete exclusion of alternative causes is not possible.

7. REPORTER INFORMATION

Reporter name	Abhishek Kumar
Profession	B.Pharm Student
Institution	Department of Pharmacy, Jagannath University, Jaipur
Contact	Academic submission record; contact details not required for this assignment.
Report date	17 September 2026
Signature	Abhishek Kumar

8. FOLLOW-UP / DOCUMENTATION LOG

Date	Source / Contact	Information / Action	Status
12 Sep 2026	Patient / clinical record	ADR identified and suspected amoxicillin documented.	Initial report
12 Sep 2026	Clinical assessment	Amoxicillin discontinued; cetirizine given; patient observed.	Action taken
13 Sep 2026	Patient follow-up	Urticaria and itching resolved; no respiratory symptoms.	Recovered
17 Sep 2026	Student documentation	Case reviewed and report completed for academic submission.	Closed

9. DOCUMENT CONTROL

Report / Reference No.	ADR-2026-001
Date received	17 September 2026
Reviewed by	B.Pharm Academic Record — Student Submission
Submission status	Completed and documented for academic submission.
Confidentiality	Patient represented by initials only; case is fictional and intended for educational use.

CONCLUSION: The documented case describes a suspected amoxicillin-associated hypersensitivity-type ADR presenting as generalized urticaria, itching and mild facial swelling. The patient recovered after withdrawal of the suspected medicine and symptomatic treatment. Based on the available case information, the causality assessment is recorded as probable/likely.

Educational reference: ADR reporting should be performed according to the current pharmacovigilance procedures and reporting requirements applicable to the institution and national authority. This completed case is a sample academic documentation exercise.